Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0016	**Age / Sex**	84 / F
Setting	Emergency Dept	**Primary dx**	Urosepsis with septic encephalopathy

Chief complaint

Acute confusion and fever for 2 days.

History of present illness

Nursing home resident, usually walks with a frame and holds conversations. Over 48 h became disoriented, agitated at night, and refused food. Foul-smelling urine reported by carers. No head injury.

Past history / medications

Mild dementia, type 2 diabetes, recurrent UTIs, urinary catheter changed 3 weeks ago. Metformin, donepezil.

Examination

Temp 38.9 C, HR 112, BP 98/54, RR 22, SpO2 95%. Suprapubic tenderness. Alert but inattentive, fluctuating attention. No meningism, no focal deficit.

Glasgow Coma Scale

Eyes open spontaneously. Speech is fluent but confused, disoriented to time and place, not at her baseline. Obeys commands.

GCS on arrival	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	4	4 - Confused
Motor (M)	6	Obeys commands
TOTAL GCS		**14**

Investigations

Urinalysis: leukocytes and nitrites positive. Lactate 3.0, CRP 190, WBC 17.5, creatinine 134 umol/L (baseline 85). CT head normal. Blood cultures taken. CXR clear.

Assessment

Sepsis from catheter-associated urinary tract infection with acute encephalopathy (delirium) superimposed on dementia. GCS 14.

Plan / disposition

Sepsis bundle with 30 mL/kg crystalloid, piperacillin-tazobactam, catheter replacement, hold metformin. Delirium care measures. Admitted to the geriatric ward, cultures guide antibiotic narrowing.